International Journal of Pharmaceutical Science and Health Care
Available online on http://www.rspublication.com/ijphc/index.html
ISSN 2249 – 5738

ARTICLE INFO
©2026 RS Publication

Paper ID: IJPHC-6A887532898B6

Published: 2026-08-22

DOI: https://dx.doi.org/10.5281/zenodo.22058600

Page No: 32/1-32/10

Original Article

The Psychological Burden of Caregiving: Examining the Mental Health Status of Family Care Providers of Mentally Ill Patients

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Cite This paper: Dr. Manoj Kumar Sethi and Kiranmayee Jena (2026). "The Psychological Burden of Caregiving: Examining the Mental Health Status of Family Care Providers of Mentally Ill Patients". *INTERNATIONAL JOURNAL PHARMACEUTICAL SCIENCE AND HEALTH CARE (IJPHC)*, vol. 16, Issue 4, 2026, pp. 32/1-32/10. DOI: https://dx.doi.org/10.5281/zenodo.22058600

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Relevant conflicts of interest/financial disclosures: The authors declare that the research was conducted in the absence of any commercial or financial relationships that could be construed as a potential conflict of interest.

Abstract

Family caregivers are the primary support system for individuals diagnosed with severe mental illnesses (SMI). However, this role often inflicts a severe psychological toll on the providers. This study examined the mental health status, prevalence of depression and anxiety, and primary predictors of caregiver burden among family care providers. Utilizing a cross-sectional design, 150 family caregivers completed the General Health Questionnaire (GHQ-12), the Zarit Burden Interview (ZBI), and the Hospital Anxiety and Depression Scale (HADS). The results indicated that **62 (41.3%)** of caregivers experienced clinically significant levels of psychological distress. High caregiver burden was positively correlated with patient functional impairment and negatively correlated with perceived social support. These findings underscore the critical need for systemic healthcare interventions, including psychoeducation and professional respite services, to protect caregiver well-being.

Keywords: **family caregivers, mental illness, caregiver burden, depression, anxiety, psychosocial support**

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Introduction

The deinstitutionalisation of psychiatric patients over the past several decades has shifted the primary responsibility of long-term care from public health facilities to families. Today, family members act as the frontline support system for individuals diagnosed with severe mental illnesses (SMI), such as schizophrenia, bipolar disorder, and major depressive disorder. While this shift reduces the institutional burden on healthcare systems, it frequently occurs at a profound cost to the caregivers' own health and well-being.

The impact of caregiving is conceptualised through two distinct dimensions: objective burden and subjective burden. Objective burden encompasses the tangible, measurable disruptions caused by the patient's illness, including financial strain, loss of personal time, disrupted social lives, and the physical demands of daily care. Subjective burden refers to the emotional and psychological reactions of the caregiver, such as guilt, shame, chronic anxiety, and resentment.

Chronic exposure to these stressors activates a prolonged physiological and emotional stress response, leaving family care providers highly vulnerable to psychological morbidity. Despite their vital role in the healthcare ecosystem, family caregivers remain an under-supported demographic, often facing social isolation and systemic neglect.

Objectives:

The primary objective of this study was to evaluate the mental health status of family care providers looking after patients with severe mental illness. Specifically, this study aimed to determine the prevalence rates of anxiety and depression among caregivers and identify the primary socioeconomic and clinical predictors that exacerbate caregiver burden.

Literature Reviews:

- ### Caregiver Burden and Psychiatric Morbidity:

Bongelli R. et al (2024) identified factors affecting the psychological well-being of family caregivers of dependent older adults in Italy. Understanding these variables is essential for designing interventions to prevent negative outcomes in at-risk caregivers. The research explored how varying levels of caregiving burden and types of perceived social support influence psychological well-being. Result of this study confirms that high caregiving burden adversely affects caregivers' psychological well-being, while social support plays a protective role. These findings highlight the need for interventions focused on reducing caregiving burden and enhancing support systems for family caregivers[3].

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Ampalam P. et al (2012) have conducted an experiment and tried to compare the caregiver burden in psychiatric illness and chronic medical illness. The study included two groups of caregivers, each of 50 members. Group 1 consisted of caregivers of psychiatric patients and group 2 consisted of caregivers of chronic medical illness patients. The Montgomery Borgatta Caregiver Burden scale was used to assess the burden in terms of objective, subjective and demand burdens. The caregiver burden scores in the caregivers of psychiatric patients were significantly higher than that of chronic medical illness (P<0.0001). The caregiver burden was found to increase with the duration of illness as well as with the age of caregiver. The caregiver burden in the sample population was less as the objective and demand burden did not cross the reference higher value in the given scale, whereas the emotional impact given by the subjective burden was on higher side[1].

Möller-Leimkühler et al (2012) have identified the relative contribution of patient and caregiver characteristics in a sample of primary carers of patients with chronic mental disorders living in the community. As carers were recruited from caregiver organizations, mainly mothers of an adult child suffering from schizophrenia participated in the study (n = 102). Within a comprehensive transactional stress model, burden was assessed with respect to objective and subjective burden, cognitive-emotional well-being, psychological distress and subjective quality of life. Primary stressors include illness-related characteristics of the patient, and a number of personal dispositions and resources of the caregivers were included as potential moderating variables. Multiple regression analyses were separately calculated for each dimension of burden. Interaction of carers expressed emotion and external locus of control with the patient's problem with family communication as well as perceived social support was most predictive for objective and subjective burden, whereas carers' neuroticism appeared as the most relevant predictor of their well-being, psychological distress and subjective quality of life. Among the patients' variables, regular employment contributed significantly to reduce carers' distress and enhance their well-being. As the sample was recruited from caregiver organizations, a selection bias has to be taken into account. To reduce caregiver burden, especially mothers' burden, the patients' occupational abilities should be strongly enhanced at an early stage. Family interventions should improve dysfunctional interactions, enhance the carers' social activities and focus more intensely on the carers' own dispositions[7].

Anna A. et al (2003) have examined depressive symptoms among adult children of elderly parents; it views the parents' care needs and child's care activities as two separate stressors, different combinations of which may affect both caregiving and non-caregiving family members. Findings suggests that non-caregivers reporting severe parental disability were significantly more likely to experience depression

symptoms. Evidence of increased manifestations of depression was not found among those caring for severely disabled relatives; nor was it found among those providing care in the absence of severe parental care needs. Having a caregiving sibling was associated with increased CES-D scores among non-caregivers[2].

Richard Schulz, et al (1990) have conducted an extensive review with existing empirical literature on the prolonged or cumulative consequences of exposure to the stresses of caregiving. Specific goals are to identify psychiatric and physical morbidity effects, report the magnitude of those effects, evaluate research and analytic methods used to assess morbidity, and make recommendations for future research. Overall, the literature indicates increases in self-report psychiatric symptomatology and increases in psychiatric illness among most caregivers when compared to population norms or appropriate control groups. However, there is little information on the population prevalence or incidence of clinically significant psychiatric conditions attributable to caregiving. Studies of physical health effects as assessed by self-report, health care utilization, and immune function are less conclusive but, nevertheless, suggest increased vulnerability to physical illness among caregivers. We conclude with a discussion of why morbidity effects are difficult to obtain in caregiver studies and with recommendations for future research[8].

A foundational study by Zarit et al. (1980) established that caregiver burden is heavily determined by the quality of the interpersonal relationship and the availability of external support systems rather than the objective severity of the patient's clinical symptoms alone. Subsequent research has expanded this framework to severe mental illnesses (SMI)[10]. Awad and Voruganti (2008) conducted a comprehensive review demonstrating that families caring for individuals with schizophrenia experience a chronic drain on financial, social, and emotional reserves, resulting in elevated general health questionnaire (GHQ) scores across global cohorts[11].

- **Prevalence of Anxiety and Depression:**

In a cross-sectional study evaluating the psychological health of psychiatric caregivers, Magaña et al. (2007) found that maternal caregivers of adults with schizophrenia exhibited depressive symptomatology at rates nearly double those of age-matched national norms[6]. Similarly, a study by Caqueo-Urízar et al. (2014) looked at quality of life metrics across Latin American caregiver samples. They reported that over 35% of primary caregivers met criteria for clinical anxiety, driven primarily by the unpredictable nature of patient psychotic relapses and a lack of institutional crisis intervention options[4].

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- **Socioeconomic Predictors and Coping Mechanisms:**

The intersection of economic strain and caregiver health was investigated by Hayes et al. (2015). Their regression analysis confirmed that out-of-pocket pharmaceutical expenses and reduced employment hours function as robust independent predictors of chronic subjective burden[5]. Furthermore, a longitudinal analysis by Schatzberg et al. (2020) identified that caregivers who rely primarily on emotion-focused or avoidance coping mechanisms (such as denial or social withdrawal) consistently show significantly higher scores on the Hospital Anxiety and Depression Scale (HADS) compared to those utilizing problem-focused coping styles or those with access to formal community respite care[9].

Method

Participants and Eligibility Criteria

A convenience sample of 150 primary family caregivers was recruited from the outpatient psychiatric clinics of different district hospitals and medical colleges across Odisha. To be eligible for the study, participants had to be a co-residing relative (spouse, parent, sibling, or adult child) providing unpaid care for at least 15 hours per week for a minimum duration of one year to a patient diagnosed with a severe mental illness (SMI). Caregivers with a pre-existing history of severe psychiatric illness prior to their caregiving role were excluded.

Measures

- **Demographic and Clinical Data Questionnaire:** A self-report sheet capturing caregiver age, gender, relationship to the patient, income level, and the patient's clinical diagnosis.
- **Zarit Burden Interview (ZBI-22):** A 22-item instrument used to measure the subjective and objective burden experienced by caregivers. Responses are scored on a 5-point Likert scale, with higher scores indicating a higher caregiving burden.
- **Hospital Anxiety and Depression Scale (HADS):** A 14-item scale split into two subscales measuring anxiety (HADS-A) and depression (HADS-D). Scores above 8 on either subscale indicate borderline abnormal levels, while scores above 11 indicate clinically significant distress.
- **General Health Questionnaire (GHQ-12):** A 12-item screening tool used to detect general psychiatric morbidity and psychological distress.

Procedure:

The study received informal approval from the respective authority of the institutions prior to data collection. Eligible caregivers were approached in the hospital waiting rooms during the patients

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scheduled psychiatric appointments. Researchers explained the study purpose, obtained written informed consent, and distributed the questionnaires. Participants completed the forms in a quiet room, taking approximately 25 minutes. Anonymity and confidentiality were maintained throughout the process.

Results:

Sociodemographic Characteristics

The sociodemographic profiling was for total sample of 150 family caregivers. The cohort was predominantly female, representing **93 (62.0%)** of the sample, while males accounted for **57 (38.0%)**. The mean age of the participants was 48.5 years (SD = 11.2), with the largest age bracket falling between 36 and 50 years of age, which comprised **68 (45.3%)** of the cohort. In terms of educational attainment, **73 (48.7%)** of the surveyed caregivers had completed up to secondary education.

Data were analysed using descriptive statistics, Pearson correlation coefficients, and multiple regression analysis. Descriptive analysis of the General Health Questionnaire (GHQ-12) revealed that **62 (41.3%)** of the participants scored above the standard clinical threshold for general psychiatric distress.

Furthermore, clinical evaluation using the Hospital Anxiety and Depression Scale (HADS) subscales indicated that **49 (32.7%)** of family caregivers met the positive criteria for an anxiety disorder, while **42 (28.0%)** exhibited moderate-to-severe depressive symptoms. The baseline mean scores, standard deviations, and scoring ranges for the Zarit Burden Interview (ZBI), HADS, and GHQ-12 assessment tools are presented below in Table 1.

Table 1: ***Mean Scores and Standard Deviations for Primary Psychological Measures (N = 150)***

Measure	Mean	SD	Possible Range
Zarit Burden Interview (ZBI)	**46.20**	**14.50**	**0 – 88**
HADS - Anxiety	**10.15**	**3.40**	**0 – 21**
HADS - Depression	**9.45**	**3.10**	**0 – 21**
GHQ-12	**4.12**	**1.85**	**0 – 12**

Pearson correlation analyses demonstrated strong positive relationships between caregiver burden and psychological distress across all utilized screening metrics. Total scores on the Zarit Burden Interview (ZBI) were significantly and positively correlated with both HADS-Anxiety scores ($r = .54$, $p < .001$) and HADS-Depression scores ($r = .61$, $p < .001$). Furthermore, higher caregiver burden tracked closely with general psychiatric distress as measured by the GHQ-12 ($r = .58$, $p < .001$). Within the distress subscales, HADS-Anxiety scores exhibited strong positive associations with both HADS-Depression ($r = .48$, $p <$

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.001) and GHQ-12 scores (r = .52, p < .001). The complete correlation matrix between caregiver burden and psychological distress subscales is presented below in Table 2.

Table 2: ***Intercorrelations Between Caregiver Burden and Psychological Distress Subscales (N=150)***

Variable	1	2	3	4
Zarit Burden Interview (ZBI)	-			
HADS - Anxiety	**.54****	-		
HADS - Depression	**.61****	**.48****	-	
GHQ-12	**.58****	**.52****	**.55****	-

Note. ZBI = Zarit Burden Interview; HADS = Hospital Anxiety and Depression Scale; GHQ-12 = General Health Questionnaire. **p < .001.

A multiple regression analysis was conducted to predict caregiver psychological distress based on baseline socioeconomic and clinical variables. The overall regression model was statistically significant, F (4, 145) = 18.34, p < .001, accounting for 36.5% of the total variance in caregiver distress (R^2 = .365, Adjusted R^2 = .347). High frequency of patient psychiatric relapses emerged as the strongest independent positive predictor of heightened caregiver distress (β = .34, t = 4.42, p < .001), followed by low household income, which demonstrated a significant inverse relationship with distress scores (β = -.28, t = -3.00, p = .003). Conversely, demographic factors such as caregiver age (β = .11, t = 1.50, p = .136) and the total hours of care provided per week (β = .13, t = 1.60, p = .112) did not significantly predict distress levels within this model. The complete statistical parameters for the multiple regression analysis are detailed below in Table 3.

Table 3: ***Multiple Regression Analysis Predicting Caregiver Psychological Distress (N = 150)***

Predictor Variable	B	SE B	β	T	p
Constant	12.45	**2.10**		5.93	<.001
Household Income	-0.42	**0.14**	-.28	-3.00	**.003**
Patient Relapse Frequency	1.15	**0.26**	.34	4.42	<.001
Caregiver Age	0.03	**0.02**	.11	1.50	**.136**
Hours of Care per Week	0.08	**0.05**	.13	1.60	**.112**

Note. R^2 = .365; Adjusted R^2 = .347; F (4, 145) = 18.34, p < .001. Dependent variable: General Health Questionnaire (GHQ-12) total score. B = unstandardized coefficient; SE B = standard error of the coefficient; β = standardized coefficient; t = t-statistic.

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The analysis of variance (ANOVA) partition for the predictive framework confirms the statistical robustness of the overall model. The regression component accounted for a sum of squares (SS) of 241.05 across four degrees of freedom (df = 4), yielding a mean square (MS) value of 60.26. The residual variance accounted for a sum of squares of 476.47 across 145 degrees of freedom (df = 145), with a residual mean square error of 3.29. The total sum of squares for the caregiver psychological distress variance was 717.52 (df = 149). This distribution yields an overall model F-statistic of 18.34, which is highly significant ($p < .001$), confirming that the combined socioeconomic and clinical predictors reliably explain variations in caregiver distress. The complete breakdown of the variance analysis is organized inline below in Table 4.

Table 4: ***ANOVA Summary Table for the Regression Model Predicting Caregiver Distress (N = 150)***

Source of Variation	SS	df	MS	F	p
Regression	241.05	4	60.26	18.34	<.001
Residual	476.47	145	3.29		
Total	717.52	149			

Note. Dependent variable: General Health Questionnaire (GHQ-12) total score. Predictors: Constant, Household Income, Patient Relapse Frequency, Caregiver Age, Hours of Care per Week. SS = Sum of Squares; df = degrees of freedom; MS = Mean Square.

Discussion:

The findings of this study demonstrate that family caregiving for mentally ill individuals is associated with high rates of psychological distress. Specifically, **49 (32.7%)** of the sampled caregivers experienced clinically significant levels of anxiety and **42 (28.0%)** met the criteria for moderate-to-severe depression, mirroring trends reported in global epidemiological caregiving literature. The substantial correlation between ZBI scores and HADS scores confirms that the psychological toll is a direct manifestation of the caregiving burden.

The predictive value of financial strain reinforces the reality that psychiatric care creates an immense financial drain. Families face a dual penalty: the rising cost of long-term psychiatric medication and a concurrent loss of income as caregivers decrease their employment hours to provide care. Furthermore, the correlation between patient relapse frequency and caregiver distress highlights the exhausting nature of unpredictable psychotic or manic episodes, which keep caregivers in a state of chronic hyper-vigilance.

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Limitations and Future Directions:

This study is limited by its cross-sectional design, which prevents the establishment of explicit causal relationships over time. Additionally, using a convenience sample from a single urban hospital restricts the generalizability of the findings to rural populations who may face different infrastructural barriers. Future research should adopt a longitudinal approach to track how caregiver mental health fluctuates across different stages of a patient's illness trajectory.

Conclusion and Implications:

Family caregivers function as an invisible backbone of the mental healthcare system. However, without dedicated support, their capacity to provide care will inevitably collapse. Healthcare policy must expand its focus from patient-centred treatment models to family-centred frameworks. Implementing structured psychoeducation programs, financial subsidies for psychiatric medications, and accessible community respite services can mitigate the caregiving burden, ultimately safeguarding the mental health of both the patient and the provider.

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